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HAVEN PAEDIATRIC CENTRE

Going into Haven

An engagement guide for the operations lead

01 Where you sit
02 Read this first. The sensitivities
03 What Haven is
04 What we found
05 What you own at Haven
06 Your first ninety days
07 How we work

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CIRCULATION
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A letter before you start

Dear Dr Omotanwa,

When I asked you to come into this I told you the job is bigger than it looks from the outside and smaller than it feels once you are inside it. Both of those are true, and the gap between them is where most new operators lose their first year. This guide and the handbook that comes with it exist to close that gap faster than experience alone would close it.

You are going into Haven as our person. You run operations there on our mandate, and if the transformation lands the way it should, you are the prospective Head of Operations when it closes. That order matters and you should hold it clearly in your head, because it changes who you answer to and what you can say. Until then Haven is a client, I am accountable for what we do there, and you carry our name into a building we do not own.

You are coming in green. I know that, and it is not the thing I am worried about. Operations is a craft rather than a body of knowledge, and the only people who learn it are the ones put in front of a real hospital with real money moving through it and expected to make decisions. What I look for instead is whether someone can hold two things at once: the discipline to run a system by numbers, and the judgement to know when a number is telling them something untrue. You can do both. That is why you are reading this.

What you should understand about Haven before you walk in is that it is a good hospital with young systems. That combination is rarer and more hopeful than it sounds. We went through the place in August and the thing that came back strongest was the staff. Sixteen of seventeen rated the safety of care very good or excellent, in an anonymous survey where they had every opportunity to say otherwise. The hardest thing to build in a hospital, a team that genuinely cares, is already built here. What is not built is everything that lets a caring team scale.

Read section 2 before you speak to anybody at Haven. On this engagement how we carry ourselves matters as much as what we find, and there is one thing in it you must not get wrong.

The honest warning is about pace. The board wants to move from roughly N18 million a month to the mid forties inside six months, and the paediatric high season from November to February is the window that makes it arithmetically possible. You arrive into that window rather than ahead of it.

My standing offer is that you can call me about anything, including the things you think you should already know, and I would much rather you called twice than guessed once. The one thing I ask in return is that you tell me early when something is going wrong. Nobody has ever been in trouble with me for bringing bad news on a Tuesday. People have got into trouble for bringing it on a Friday when it had been true since the previous Tuesday.

Read the handbook properly. It is written to be worked through rather than read, and there are places in it where I have left you space to write. Use them.

Dr Debo Odulana

Founding Partner, Consult for Africa

01 Where you sit

You are a Consult for Africa consultant placed into Haven to run the facility day to day. The hospital is a client of ours, on a transformation engagement that runs in four tracks: the diagnostic audit, which is done; culture and clinical standards of work; process and operations; and revenue and growth.

Who	What they hold
You	Operations at Haven, day to day. The roster, the tariff and capture, stock, the cash cycle, the front desk, the data, the standards of work.
Me	Partner oversight, the board relationship, the commercial conversation, and anything touching the sensitivities in section 2.
Tito Ipinmoye	Director of Product and Strategy. She led the August fieldwork and knows Haven's record better than anyone outside it. Go to her for what we actually saw.

Route these to me, always: anything about money or payment, anything involving the board or the owners' relationships, anything that changes the scope of what Haven is trying to do, and anything in section 2.

What "prospective" means

Post-transformation you are the candidate to take the Head of Operations seat at Haven itself. That is the intention and it is not a commitment either way yet, by you or by them. Until it is settled, you are ours, you are paid by us, and the standard you hold is ours. Do not let anybody at Haven imply otherwise, and do not imply it yourself.

02 Read this first. The sensitivities

This is not a normal turnaround. Internalise this section before you speak to anyone at Haven.

The incident, and how we hold it

In May 2026 Haven lost a child. The child was referred in already critically ill, was in a very poor state, and may not have been saveable under any circumstances. The loss is not attributable to the facility. It is what prompted the leadership to call us.

We do not raise it. Ever. Not in writing and not in conversation. Not in a meeting, not in an interview, not in a WhatsApp message, not in a note to file. If a member of staff raises it with you, listen with care, do not probe for blame, and move on.

Every client-facing word is forward-looking. We are here because a strong young facility is putting proper systems under its growth, not because something went wrong. That is the honest framing as well as the sound one, and the operational case stands entirely on its own without ever invoking the loss.

Why it is this strict: documents travel. A report, a note or a message is discoverable, and a written record linking a death to any gap in a facility is a liability for Haven and for us, and it reopens a wound for a grieving board. One of the owners is a former co-founder of mine.

> If you ever find yourself about to write a sentence that connects the May loss to anything you have found, stop and call me instead. There is no version of that sentence that helps.

The relationship

I was invited to Haven's board from inception but do not sit on it, and the board is not formally constituted. Say "invited to the board", never "sits on the board". Dr Shakirah Saliu is a former co-founder of mine from Doctoora. We are a paid adviser to a company I am close to, which we handle with complete transparency and no special treatment in either direction. You do not have to manage any of that, but know it, and never imply leverage.

The commercials

The fee is fixed, already heavily discounted from our standard, and carries no success fee, which keeps the related-party position clean. You do not need the figures and they are not yours to discuss. If payment comes up in any form, it comes to me.

The thesis, which is ours

This is a culture and systems build, not a set of process patches. Strong routines, standards of work and incentives are what make safe, efficient care self-sustaining. Culture is the multiplier. Everything we recommend ladders up to that, and it is the line to use.

03 What Haven is

A private children's hospital in GRA Ikeja, a little over a year old. Five general paediatric beds and a three-cot neonatal intensive care unit, with an outpatient clinic, a pharmacy, a small laboratory and a theatre that is not yet working at the level it could.

OPENED	May 2025, so a little over a year trading
BEDS	5 general paediatric, 3 NICU cots
CURRENT REVENUE	Roughly N15 to 20 million a month
TARGET	N45 to 50 million a month at an 18 to 20 per cent EBITDA floor
WINDOW	The paediatric high season, November to February

It does not deliver babies. That single fact shapes more of your job than anything else on that list, because it means the NICU, the highest-yield asset in the building, cannot fill itself from the hospital's own maternity activity. Every baby in those cots was referred in by somebody who chose Haven. Referral is therefore not a marketing nicety here. It is the supply line for the most valuable thing Haven sells.

The owners

Who	What to know
Kabir Aregbesola	Lead board contact, carries the commercial relationship.
Mrs Abisodun Alli	Founder. Use Abisodun Alli; Alli is her maiden name and the one she uses.
Dr Shakirah Saliu	Founder. Note the h in Shakirah. See section 2.
Dr Odedina	Consultant neonatologist and Medical Director. The NICU rests on him clinically, which makes him the most important single relationship you will build.
Mr Ogochukwu Odum	Founder.

Four of the five completed a direction survey. They are aligned on reinvesting rather than taking money out, professionalising, perfecting this one site before opening another, and backing the staff. Where they differ is how high to climb and how fast.

04 What we found

TIED UP
N7M
N4.2m in HMO receivables, N2.77m in pharmacy stock

OF THE TARIFF
80.9%
1,952 of 2,413 lines priced at zero

BELOW MARKET
34%
on the prices that do exist

The N7 million is the one people misread. It is not a loss and it is not missing. It is Haven's own money sitting on a pharmacy shelf and in two HMOs' ledgers, mostly Leadway and NEM, where the receivable is close to a full period's revenue. "There is barely anything left after salaries" is a working capital problem wearing the costume of a profitability problem, and the two have completely different cures. Confusing them is the most common error made by people in your seat, and the handbook spends a whole section on it.

The tariff finding is the fastest money in the business. Pricing the unpriced lines against a real peer hospital's schedule, and closing the gap to market in two tranches rather than one jump, is worth at least N2 million a month before a single extra patient walks in. It needs no equipment, no hire and no building.

The reporting finding is the one that makes your seat necessary. A hospital that cannot reconcile its own visit counts cannot be managed by anyone, however good. Until the record is trustworthy, every decision above you is being taken on feel.

The audit report is yours to use as a reference rather than a read-through. It is organised by domain, so go to the domain and find what we found. Everything in it is held to the framing rule in section 2.

05 What you own at Haven

Four of the ten growth levers are substantially yours to deliver rather than to support: repricing the tariff and fixing capture, freeing the working capital, getting the NICU running at high occupancy operationally, and the standards, roster and incentive work that lets the culture survive growth. The handbook takes each in turn and says which of the other six you merely enable.

The clinical decisions remain clinical and remain with Dr Odedina and the medical team. Section 15 of the handbook is specific about that boundary, because getting it wrong is the fastest way for an operations lead to lose a clinical team's confidence, and you will not get it back.

The thing nobody will tell you directly

Your first real test is not operational. It is whether a hospital full of people who have been running the place by instinct since it opened will let a newcomer change how they work, and you arrive carrying a consultancy's name, which makes it harder rather than easier. You will arrive with a plan and they will have a hospital. Both are legitimate. Read section 21 of the handbook before your first week rather than after it.

06 Your first ninety days, in outline

Window	What matters
Weeks 1 to 2	Understand before you change. Walk every shift including nights. Learn the names. Find out where the record actually lives as opposed to where it is supposed to live. Change nothing structural.
Weeks 3 to 6	The tariff and the capture. The fastest money and the clearest early win, and doing it teaches you the hospital's clinical activity better than any briefing could.
Weeks 5 to 8	The cash cycle. Push the two large HMO balances, put the monthly receivables discipline in, and move the pharmacy towards consignment.
Weeks 7 to 12	The rhythm. Install the daily huddle, the weekly operating review and the monthly pack, so that by the end of the quarter the hospital can see itself.

Underneath all twelve weeks the NICU readiness and referral work runs, because the high season does not wait for your onboarding to finish.

07 How we work

Daily contact with me during your first fortnight, then as needed, and always when something is going wrong. A short written note before each board working session: what moved, what did not, what you need decided, kept to a page. The board sees our work through you, so what you send is the firm's output and not just yours.

We are collaborative rather than audit-by-ambush. The Haven team is a partner in the findings, never a target. We diagnose a system, never a person. Partial or messy records are themselves a finding, so take what exists and do not ask anybody to reconstruct things to make us comfortable.

What I expect of you

I expect you to know your numbers without looking them up. Section 2 of the handbook lists the ones I mean, and I will ask you for them.

I expect you to tell me things early, in the state they are actually in, including when they reflect badly on you. I have never minded being told something is broken. I mind being told late.

I expect you to protect the staff survey result. Everything else in that hospital can be rebuilt if you get it wrong. That cannot, not quickly.

And I expect you to argue with me. You will be in the building and I will not. If the plan is wrong in a way that is only visible from inside, you are the only person who can see it, and a quiet operations lead who implements a flawed plan faithfully is worse than useless to me. Say so, say why, and bring what you would do instead.

Welcome to the firm, and to Haven.